

Medical Record Abstraction for Breast Cases

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14.1 Overview

As noted in Chapter 11 (Breast Tissue Biopsies), breast screening is not common in Ghana and most women present with palpable lumps. The majority of patients receiving breast biopsies will be diagnosed with invasive cancers.

This chapter describes the procedures for obtaining and documenting diagnostic information for study Cases using the Breast Case Abstract Form (See Appendix 14-1, Breast Case Abstract Form and Appendix 14-2 instructions for completing the form). Cytologic and histologic diagnoses will always be rendered by the pathologist according to routine procedures for patient management purposes. Study staff may need to obtain copies of the cytology or the biopsy pathology report, and possibly other materials, and transcribe them onto a study data collection forms. For women diagnosed with invasive cancer who go on to surgery, a Surgical Pathology Report will be completed by the pathologist to document the macroscopic and microscopic findings from the surgical procedure. Information from the Surgical Pathology Report will be captured in Section D of the Case Abstract Form. Lastly, the NCI study pathologist, Stephen Hewitt, will conduct additional histopathologic reviews for research purposes only. All of these activities are described in this chapter in more detail.

14.2 Breast Case Abstract Form

The following section describes the information to be collected on the Breast Case Abstract Form.

Diagnosis information, Section A, is likely to be available much earlier than the information requested in the rest of the form. Begin to complete the Diagnosis Information 6 weeks after enrollment and continue to search for and record this information until a definitive diagnosis is made. Then that section of the form can proceed to data entry and be entered in unsubmitted status until the rest of the form can be completed. The information in Sections B through D may take up to 1 year before they are available. When available the entire form should be entered and submitted as complete.

14.2.1 Section A. Diagnosis Information

This section captures the date the breast cancer was first suspected and all procedures performed to confirm the disease, including the date, place, type of procedure, and diagnosis. This information should be captured for all women indicated to have a biopsy as well as those women with a diagnosis of cancer who come to the participating hospital for treatment. If feasible, please record the information in chronological order from the earliest to the most recent diagnostic procedure. If this is too time consuming based on the way the information are arranged in the medical records, record the diagnostic procedures in any order, but be sure to record the procedure dates accurately.

14.2.2 Section B. Treatment Information

For women diagnosed with cancer and whose diagnosis information is documented in Section A of the Breast Case Abstract Form, treatment information is collected in Section B of the form. This section captures all treatment information, including the date, place, type of treatment, and type of surgery (if applicable).

14.2.3 Section C. Receptor Status

For women diagnosed with invasive breast cancer, whenever possible and available, information on receptor status will be collected in Section C of the Breast Case Abstract Form.

Table 14-1 presents guidelines for which sample information to use for documenting the receptor status on the Breast Case Abstract Form depending on whether the case has undergone surgery. When documenting receptor status on the Breast Case Abstract Form, the receptor status from surgery should always be used above all other procedures, regardless of whether the patient received neo-adjuvant chemotherapy. When surgery receptor status is not available, then biopsy or fine needle aspiration (FNA) information can be used instead.

Table 14-1. Guidelines for Determining Receptor Status

FNA/Biopsy	Surgery at participating hospital		
	No	Yes	
		No Neo-adjuvant Chemotherapy	Neo-adjuvant Chemotherapy
FNA Only	Complete Section C, Receptor Status, of form for ER, PR, and HER2 based on FNA, if available.	Complete Section C, Receptor Status, of form based on surgery specimen (i.e. mastectomy, lumpectomy, or wide-local excision) for ER, PR, and HER2, if available.	Complete Section C, Receptor Status, of form for ER, PR, and HER2 based on biopsy specimen if available.
Needle Core, Excisional, Incisional, or Punch Biopsy	Complete Section C, Receptor Status, of form for ER, PR, and HER2 based on biopsy specimen if available.		

14.2.4 Section D: Surgical Pathology

This section of Breast Case Abstract Form was developed in collaboration with study pathologists in order to collect information related to obtaining and documenting diagnostic information for research purposes.

At all hospitals, standard procedures are followed to render a diagnosis after biopsy or surgery. The turnaround time for a pathologic diagnosis is generally one to two weeks. There are currently two study Pathologists each at KATH and KBTH.

For the purposes of the study, the primary interest is to obtain diagnostic information derived from biopsy and surgery (when available). A copy of the report documenting the initial cytology or pathology diagnosis from a biopsy should also be obtained by the study staff to confirm breast cancer in cases where a surgical report cannot be obtained or when additional information is deemed to be necessary by the pathologist.

Table 14-2 presents guidelines for obtaining a cytology, biopsy pathology, and/or surgical pathology report for purposes of the study. Which reports are to be obtained will depend on the diagnostic procedures performed as well as on the location where they were performed.

To summarize,

- 1) All patients that have surgery at a participating hospital will have Section D, Surgical Pathology, of the Breast Case Abstract Form completed by a pathologist at that hospital. A copy of the cytology or biopsy pathology report from the initial diagnosis will also be obtained, whenever possible.
- 2) A surgical pathology report will be requested for all patients who undergo surgery at a non-participating hospital and Section D, Surgical Pathology, of the Breast Case Abstract Form will be completed by study staff based on that report.
- 3) For patients who do not undergo surgery, or for whom a surgical report cannot be obtained, the staff will obtain the cytology or pathology report from the initial FNA or biopsy.

Table 14-2. Guidelines for Obtaining Diagnostic Reports

FNA/Biopsy		Surgery		
		No	Yes	
			Participating hospital	Non-participating hospital
FNA Only	Participating hospital	Request/obtain cytology report	Complete section D, Surgical Pathology, of case abstract form based on surgical pathology	1. Request/obtain surgical pathology report and complete Section D, Surgical Pathology, of case abstract form based on surgical pathology 2. Request/obtain biopsy pathology report, if necessary
	Non-participating hospital			
Needle Core, Excisional, Incisional, or Punch Biopsy	Participating hospital	Request/obtain biopsy pathology report	1. Request/obtain biopsy pathology report 2. Complete Section D, Surgical Pathology of case abstract form based on surgical pathology	
	Non-participating hospital		1. Complete Section D, Surgical Pathology, of case abstract form based on surgical pathology 2. Request/obtain biopsy pathology report, if necessary	

14.2.4.1 Completing Section D, Surgical Pathology, of Breast Case Abstract Form

Pathologists at participating Ghana Breast Health Study hospitals have agreed to complete Section D of the Breast Case Abstract Form. This section allows for the collection of key information such as the macroscopic (gross) description of the tumors as well as diagnostic classification of breast tumors in a standardized format.

The pathologist will complete Section D during the review of surgical specimen slides from study participants. Section D will only be completed once for the most invasive/largest tumor. No supplemental D sections will be provided.

14.2.4.2 Timing of Completion of Section D, Surgical Pathology, of the Breast Case Abstract Form

Section D of the Breast Case Abstract Form will be completed by the responsible pathologist immediately following his or her review of the microscopic slides of surgical specimens.

14.3 Disposition of Forms

On a weekly basis, the study center staff will arrange to pick up the completed Breast Case Abstract Forms from the pathology laboratory. Forms will be reviewed for legibility and completeness. Any questions about the forms will be directed to the pathologist who completed the form or another pathologist, as appropriate. Once the forms are received at the study center, the forms will be reviewed by the data entry staff and entered into the data entry system.

14.4 Obtaining the Surgical Report from Non-Participating Hospitals

If the surgery for a study participant is done at a non-participating hospital, the study staff will attempt to request a copy of this report and use the information on the report to complete the Breast Case Abstract Form. In this situation, instead of the pathologist name, the staff member who completes the form should insert her 3 digit identification code in Field #24 (Pathologist Name).

14.5 Obtaining the Cytology and/or Pathology Report

In cases in which surgery was not performed, or when a surgical report from a non-participating hospital could not be obtained, the study staff will try to obtain a copy of the cytology or the biopsy report which formed the basis of the initial diagnosis for completion of the Case Abstract Form Section A: Diagnosis Information.

14.6 Other Histopathologic Review by Study Pathologist

Dr. Stephen Hewitt from the NCI will also be conducting additional reviews of pathologic material and data collected in Ghana. If there is interest, some of these cases could be discussed with the Ghanaian pathologists at a multi-headed microscope at a future date for academic purposes.